

October 27, 2017

**CONFIDENTIAL – For Settlement Purposes Only**

Ms. Misti Jenkins  
 Mohave Transportation Insurance Company  
 2200 S. 75th Avenue  
 Phoenix, AZ 85043

OCT 30 2017

RE: Your Insured: Tamirat Erichafo  
 My Client: Cassandra Henze  
 Date of Loss: September 12, 2016  
 Claim No.: 1609130258332 -B ssm

Dear Ms. Jenkins:

As you are aware, on September 12, 2016, my client, Cassandra Henze, was a passenger of a vehicle traveling south on IL Route 47 in Newcomb Township, Champaign County. Ms. Henze's vehicle came to a stop in the left turn lane at US Route 136. At that time, your insured's driver, Tamirat Erichafo, was driving a semi-tractor and trailer north on IL Route 47, toward the same intersection. As Ms. Henze's vehicle turned left across the northbound lane of traffic, your insured's driver failed to obey the stop sign and drove directly into the rear passenger side of Ms. Henze's vehicle.

Mr. Erichafo was issued a citation for **failure to observe a stop sign**, a violation of 625 ILCS 5/11-904.

**Medical Treatment**

Immediately following the collision, Ms. Henze was fully immobilized at the scene by paramedics from Gibson Community Hospital due to complaints of pain in her neck, left knee and right shoulder. On examination, she had swelling in her left knee and tenderness in her back, left knee, and right arm. A pillow was placed under her left knee and ice pack was applied for comfort. Ms. Henze was transported by ambulance to the emergency department of Carle Foundation Hospital.

Upon arrival at the Carle Emergency Department, Ms. Henze was examined for pain in her neck, right low back, left knee and right shoulder. She reported overall pain as 6/10 with **tenderness in her neck, lumbar spine, right clavicle, and left knee**. X-rays of her neck, left knee, right clavicle, cervical and lumbar spine were obtained and reviewed. During her stay, Fentanyl (pain relief) and Cyclobenzaprine (muscle relaxer) were administered. Norco (pain relief) and Flexeril (muscle relaxer) were prescribed as discharge medications.

On September 16, 2016, Ms. Henze presented to Christine Henrichs, MD at Carle in Mahomet with ongoing complaints of pain in her right arm, right shoulder, and low back, as well as

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EXHIBIT C

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shakiness in her right shoulder, unsteadiness in her right leg when she stood, and a tingling sensation in her right shoulder with overhead activities. On examination, Ms. Henrichs had **tenderness and tightness throughout her right upper back and front right shoulder**, as well as **coldness and numbness in her right shoulder when performing overhead activities**. Dr. Henrichs advised Ms. Henze to continue her medications and prescribed Ibuprofen. She was referred for consultation with a neurologist and an MRI with instructions to follow-up in two weeks.

On September 21, 2016, Ms. Henze presented to a neurologist at Carle Spine Clinic, Zeeshan Ahmad, MD, with complaints of back pain radiating down her right buttock and right thigh. Ms. Henze experienced shaking in her right leg, which increased her pain, and she reported increased pain with physical activity, especially at work. On examination, **the movements of her low back were restricted with pain**. The X-ray of her low back was reviewed and Ms. Henze was diagnosed with **lumbar radiculitis**. Dr. Ahmad recommended her to start physical therapy and referred to a neurologist for evaluation of her right leg. Ms. Henze was advised to obtain an MRI of her back and follow-up in 2-3 weeks.

On September 30, 2016, Ms. Henze followed up with Dr. Henrichs for pain in her right shoulder. She had been taking Norco and had tried Ibuprofen, neither of which alleviated her pain. Ms. Henze complained of stiffness and tightness when she moved her right arm and performed overhead activities, numbness in her right hand while brushing her hair. On examination, she had **tightness and tingling sensation in her right shoulder when she moved her arm**. Dr. Henrichs recommended an EMG of her right upper extremities and prescribed Norco and Gabapentin.

On October 5, 2016, Ms. Henze presented to Stephen Rudawski, DO, at Carle in Mahomet. On examination, there was **tenderness and a grating sound in her right shoulder joint; prominent right-side tissue texture change**, and **tenderness in her spine area**. Ms. Henze was diagnosed with a **right AC joint sprain; thoracic outlet syndrome; and cervical, thoracic, and lumbar spine strains**. Dr. Rudawski recommended a consultation with physical therapy and prescribed Baclofen and Naproxen. Ms. Henze was told to follow-up in two weeks.

On October 24, 2016, an MRI of Ms. Henze's lumbar spine was obtained at Carle Foundation Hospital. On October 27, 2016, Ms. Henze presented to James Harms, MD, neurosurgeon at Carle, for the low back pain which increased with bending, lifting, and twisting. She had stiffness and pain in her right buttock which radiated to her right thigh. On examination, she had **tenderness in her spine, and had trouble bending forward and backward**. Dr. Harms diagnosed Ms. Henze with a back strain and recommended that she undergo physical therapy.

On November 2, 2016, Ms. Henze presented for physical therapy evaluation at Carle Therapy Services for pain in her low back and right shoulder. Ms. Henze's pain increased with bending, sitting, rising, prolonged standing and walking, and as the day progressed. Ms. Henze's pain also interfered with her physical activities and work duties. She was advised to begin physical therapy one to two times a week for four weeks, with strengthening and stabilization exercises, as well as a home exercise program.

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On December 12, 2016, Dr. Aronson performed and reviewed an EMG of Ms. Henze's right arm.

On December 16, 2016, Ms. Henze returned to Dr. Henrichs with complaints of persistent pain in her right shoulder, occasional coldness or numbness in the right hand, and significant pain when moving her right arm. On examination, Ms. Henze had **pain when moving her right arm and pain in her right shoulder with trembling and numbness.** Dr. Henrichs recommended that Ms. Henze obtain an X-ray and MRI of her right shoulder and an orthopedic consultation.

On January 16, 2017, Ms. Henze was discharged from physical therapy. She had persistent pain in her central low back and right shoulder. She continued to have difficulty with bending, sitting, prolonged standing, and walking. On the same day, Ms. Henze obtained an MRI of her right shoulder, which revealed inflammation of the supraspinatus/rotator cuff tendon.

On January 19, 2017, Ms. Henze presented to Robert Gurtler, MD, an orthopedic surgeon at Carle, due to the pain and weakness in her right shoulder over the acromioclavicular joint and anterior shoulder. She had numbness and tingling sensation in her right hand, and her pain had increased and interfered with her work. Ms. Henze could not perform overhead activities or reach her back, and had experienced a popping and catching sensation in her right hand. On examination, she had **tenderness in her right shoulder joint and a deformity of her right AC joint consistent with a shoulder separation.** Ms. Henze's right shoulder movement was restricted by pain and stiffness. The MRI of her right shoulder was reviewed. A Depo-Medrol injection was administered to her right shoulder, and Ms. Henze was instructed to follow-up as needed.

On February 21, 2017, Ms. Henze presented to Dr. Henrichs at Carle Foundation Hospital for complaints of pain in her right shoulder. On examination, Ms. Henze's right shoulder pain was associated with numbness and a tingling sensation in her right arm and she had a shocking sensation which went down her back when she moved her neck.

## **Medical Bills**

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As a result of her injuries, Ms. Henze became liable for medical bills in the amount of **\$30,859.03.**

## **Past Loss of Income**

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At the time of her injuries, Ms. Henze was employed as a child care provider at Carle Caring Place and also worked in retail. Ms. Henze sustained severe injuries to her right shoulder, neck and low back. Because of the collision, she has difficulty performing overhead activities, reaching her back, and she was only able to work half-days due to her chronic pain. She was forced to miss work after the accident, and sustained lost wages in the amount of **\$385.65.**

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### **Pain and Suffering/Loss of Normal Life**

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Ms. Henze, a 26-year-old woman, lived a carefree independent life before the collision. Prior to the collision, Ms. Henze had no difficulty at her job. As a child care provider, Ms. Henze was responsible for infant care. Since the collision, Ms. Henze has had difficulty carrying infants with her right-hand due to pain and weakness. She has had difficulty with bending, sitting, rising, and prolonged standing and walking.

In addition to her difficulties at work, Ms. Henze has had difficulty performing overhead self-care activities such as brushing her hair and washing her hair. Ms. Henze has difficulty performing her household chores such as cooking and doing the laundry. While she has completed treatment, and continues to engage it at home exercises, she does continue to experience regular pain with repetitive motions while using her arm as well as grinding and popping in the joint area.

### **Conclusion**

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Enclosed you will find the provider-wise exhibits for your review:

1. Illinois Traffic Crash Report;
2. Medical Records;
3. Medical Bills and Billing Summary; and
4. Wage Loss Verification Form

At this time, I would recommend **\$160,000.00** as full and final settlement of this claim. I will look forward to your response sometime in the next 30 days.

Very truly yours,



Miranda L. Soucie

[msoucie@spiroslaw.com](mailto:msoucie@spiroslaw.com)

MLS/eb

Enclosures

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